



OFFICIAL

Death of a Patient Procedure

1. Scope

Site	Operational Area	Applicable to
Armada Kalamunda Group	All clinical services	Nursing and Midwifery

2. Purpose

The purpose of this procedure is to ensure all deceased patients are prepared for transfer to the mortuary or funeral home as soon as practical after death.

In the event of mass casualties, refer to the Business Continuity Plans.

Patients valuables are only to be released to the next of kin (NoK) nominated on the admission sheet. The NoK must sign receipt for valuables.

Armada Kalamunda Group is not responsible for the safekeeping of jewellery left on the patient at the request of relatives - the NoK should be notified of this and the interaction documented in the health record.

All requests to view deceased patients are to be referred to the Hospital Nurse Manager. While the health service will attempt to be flexible, family may be requested to view the deceased at the funeral home. Requests to view by persons outside of the immediate family will be referred to the next of kin.

Kalamunda Hospital does not have facility to view patients in the mortuary.

3. Specific requirements when caring for a Deceased Patient

3.1 Psychiatric patients

Any death which occurs within an authorised mental health facility in Western Australia must be reported to the Chief Psychiatrist in accordance with [WA Health Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist](#) see appendix 2 for mental health notification process.

3.2 Bariatric patients

Patients weighing more than 200 kg cannot be accommodated in the Armada Health Service mortuary. Advise the Nurse Manager if the patient is unable to be accommodated in the mortuary. The Nurse Manager will contact the Funeral Home to organise collection as soon as possible. Refer to the [AKG Bariatric Management Policy](#) if required.

Before referencing this procedure document please ensure that you have the latest version from the [Policy Library](#) information hub page.

Kalamunda Hospital mortuary fridges are rated for patients up to 300kg, however bariatric patients should only be transferred to the mortuary only if there are sufficient staff for safe manual handling.

3.3 Deceased babies and miscarriages

- When appropriate deceased babies are to be taken to the mortuary and placed in mortuary fridge with entries made in the mortuary book ensuring patient labels are used.
- Miscarriages to be placed in a container from the labour ward pan room and stored in small fridge in mortuary. Complete AKMR 73.4; Management of the aftercare of 14-20-week fetus. Entries to be made in mortuary book as above ensuring patient labels are used.
- Labour Ward has a cuddlecot that can be accessed, and a body hold fridge in labour ward that can be utilised until the parent/s are discharged from the Unit.
- Further information and guidance for pregnancy loss > 14 weeks can be located on the [AKG Perinatal Loss Policy](#)
- Inform Midwifery Manager of pregnancy loss > 14 weeks via email and cc ahs.perinatallossmiswife@health.wa.gov.au

3.4 Death of a patient in custody

- If a patient dies whilst in custody the room must be sealed off and treated as a crime scene.
- Death of a patient in custody includes a child under the care of the Department for Child Protection and an involuntary patient under section 21 of the Mental Health Act 2014.
- Nothing may be touched on the patient or within the crime scene. It is likely that every death in custody will proceed to inquest.

4. Reportable death to the Coroner

- Refer to EMHS [Reporting and Care of the Deceased Policy](#).
- All unexpected deaths in an authorised unit under the WA Mental Health Act are reportable to the Coroner.
- Should difficulties arise (e.g. where there is a need to prepare the deceased patient's body for viewing by NoK), the coronial investigator is to be telephoned for guidance 9267 5700.

5. Notification of infectious diseases

- If the deceased has a confirmed or suspected infectious disease, this information is to be included in the patient's medical record and is also placed on the back of the mortuary form for the Pathologist's information (with certain diseases, post-mortems may not be performed).
- If the deceased has a confirmed or suspected diagnosis of an infectious disease, management will be coordinated by the Infection Prevention & Management (IPM) Clinical Nurse Specialist via the Hospital Nurse Manager (CNM) or Nurse Unit Manager (NUM) during office hours.
- Notifiable diseases, which require patients to be placed into a body bag:
 - Viral Hepatitis
 - HIV
 - Tuberculosis
 - Haemorrhagic Fevers

- Creutzfeldt Jacob Disease
- Anthrax
- Hepatitis B (NHMRC, 2010)
- In the case of a pandemic, body bags may run out. In this case, the deceased should be prepared as usual without the body bag.
- Covid-19. Refer to Recognising and responding to acute deterioration-Covid-19

Pandemic Influenza Patients **are not** to be placed in a body bag.

5.1 Handling of the deceased with a diagnosis of pandemic influenza

- Those handling the body should don the standard Personal Protective Equipment (PPE) i.e. plastic gown (fluid repellent), gloves, mask, and eye protection.
- Prior to moving a body with a suspected or known infectious disease, A surgical mask is to be placed on the deceased prior to movement of the body and for duration of care until the body is placed in a shroud, to minimise contamination by respiratory secretions. The body should be transferred to the mortuary as soon as possible after death.
- Prior to transport, the body should be fully wrapped in the standard hospital shroud and then placed in a body bag. There is a supply of bariatric body bags available through the HNM located in the Operations Hub.
- Regardless of whether Armada Health Service (AHS) is designated as a pandemic hospital or not, transfer/transport arrangements may be required.
- The body may be viewed during office hours only, provided visitor protocols are maintained and PPE is provided.
- If required, the Nurse Manager will liaise with the Disaster Preparedness Management Unit (DPMU) regarding transfer of the deceased. Refer to the [Business Continuity Plans](#).
- The body will be placed on a stainless-steel tray for transport purposes.
- See appendix 1

6. Documentation

- Completion of paperwork should be expedited in all instances.
- If the Next of Kin (NoK) is not present, they should be contacted by phone as soon as possible.

7. Deaths at Kalamunda Hospital

- When medical staff are on site, the death in hospital AKMR6 is to be completed by the medical staff.
- When no medical staff are on site, the Assessment of Life extinct form AKMR 8 is to be completed by two nurses, one of whom must be a registered nurse. The medical staff will then complete the AKMR6 on their next working day.
- The funeral director must be informed prior to pick up if the patient is over 120 kg.

8. Confirmation of extinction of life

- Only a Medical Practitioner may complete and sign a Certificate of Death.

- The Medical Practitioner who immediately before the death was responsible for the medical care of the deceased or who examined the body after death should certify the death and complete Life Extinct Form. The Death Certificate is to be completed by the treating team during office hours (where the death is not a Coroners Case).
- Medical staff are to complete the Death in Hospital Form and ensure this is filed in the patient's medical record. If required, a cremation certificate must also be completed.

9. Care of the deceased

9.1 Equipment

- Trolley
- Body bag
- Soiled linen carrier
- Linen and dressings
- Clothing and Valuables book, patient property bag and valuables envelope
- Mortuary box containing:
 - copy of AHS "Death of a Patient -Last Offices" Procedure
 - gown
 - comb
 - gauze bandage
 - adhesive tape
 - scissors
 - mortuary form
 - gloves
 - permanent ink pen
 - occlusive dressings
 - addressograph labels

9.2 Procedure

- Always offer relatives or identified significant others the opportunity to help with last offices.
- Ensure religious considerations are met, unless it is a Coroner's case.
- Screen area to ensure privacy. Remove excess equipment and bedding.
- Commence last offices after Medical Officer (MO) has certified patient deceased and completed the Information for Release of Bodies form (at Kalamunda Hospital after the AKMR8 is completed by 2 nurses if there is no MO present).
- Lay the person on their back, as soon as possible, head on one soft pillow and limbs straightened.
- Place dentures in mouth (if worn) and close the lips. Support the jaw by placing a rolled towel on the chest underneath the jaw.
- Close the eyelids. Should this be difficult, apply a thin wisp of moistened cotton wool on the eyelids.
- Remove all drainage tubes, e.g. tracheostomy tubes, catheters, IV cannula, unless the patient is a Coroner's case.

- If indwelling catheter in situ, drain the bladder by pressing on lower abdomen prior to its removal.
- Wash body if contaminated with body fluids and excreta.
- Re-dress and seal all wound, abrasions, intravenous sites and drain sites.
- Comb hair.
- Two staff, one of whom must be a Registered Nurse, are required to confirm the details of the patient's addressograph label (which is to be placed on the body bag and completed mortuary form) are the same as the patient's identity (ID) band on the wrist and ankle.
- Ensure patient ID bands (with addressograph inserted) are attached to both the deceased's wrist and ankle.
- Dress patient in a hospital gown or own attire (if requested by NoK).
- Place patient's addressograph label in the ID pocket of body bag.
- Document relevant information in the Nursing Progress Notes, including details of family notification, care of valuables, clothes and prostheses.
- Arrange transport of body to the mortuary, by contacting PCA/PSA
- Ensure the PCA has the required information to complete Mortuary Register (addressograph label and mortuary form)
- The body will not be released without an identification band and mortuary label, except in cases of death with a diagnosis of Pandemic Influenza (PI).

10. Valuables

- All jewellery is removed. The only exceptions are if it is not possible for the wedding ring to be removed, it is to be sighted by two nurses, covered with adhesive tape and recorded on the mortuary label. Nursing staff are to inform the PCA (who transports the body to the mortuary) of the jewellery and PCA will counter sign the mortuary label.
- The number of items and a description of removed valuable items is recorded.
- on the front of the valuable's envelope/clothing and valuable sheet, checked and signed by two persons, one being a Registered Nurse. Security is then to be notified to collect to store in the hospital safe. Kalamunda coordinator will arrange for placement in hospital safe.
- Document clothing in Clothing and Valuables Book, store on ward and call NoK to collect.

11. Transfer of a Patient to the Mortuary

- Some deceased patients may go directly from the ward to the funeral home if time and conditions related to the cause of death permit.
- All deaths in the hospital must be reported to the Hospital Nurse Manager (HNM) on duty by telephone.
- The funeral director is to be informed of any patient weighing over 120kg.

It is a legal requirement that the mortuary register be completed correctly whenever a deceased is admitted or discharged from the mortuary.

11.1 Armadale Hospital Transfers

- Nursing staff are to call the Orderly via pager 026 to collect the Bier trolley and to transport the deceased to the mortuary. The ward PCA assists with the transfer as two staff are required in the transfer. The body is prepared for transfer as per AKG Death of a Patient Procedure.
- Where the deceased is a baby, the midwife and ward PCA transport the baby to the mortuary. Anytime a baby is taken from the mortuary, an entry is required in the mortuary register to reflect the location of the baby, and again on return of the baby to the mortuary. The management of still born babies require specific processes. Refer to the policy links at end of document.
- The orderly and ward PCA are to ensure the patient's details are recorded in the mortuary register and the deceased patient's identification sticker is adhered to the fridge door.
- When the Funeral Director arrives, the HNM on duty will page the orderly on pager 026 who will meet the Funeral Director at the back door of the mortuary.
- The orderly must confirm the identity of the deceased patient with the Funeral Director with three points of identification required by the following:
 - The patient identification label on the mortuary fridge door
 - The patient identification label on the outside of the body bag
 - The patient identification label on the wrist or ankle of the patient
- The three points of identification must be checked and matched to the mortuary register and details in the funeral directors' logbook upon release of the body. If there are any discrepancies in the identification, the Orderly is to notify the HNM.
- The orderly is to obtain a collection receipt from the Funeral Director.
- The orderly will ensure that the collection receipt is handed to the HNM on duty immediately the patient is transferred. The nurse manager will then complete the Mortality and Mortuary log in the HNM office and forward the collection slip to Medical Records for filing.
- The orderly will notify the HNM on duty of any patient they enter or remove from the mortuary.
- The Armadale Hospital mortuary can hold up to six bodies. If the mortuary is full the Hospital Nurse Manager will arrange for allocated Funeral Directors to attend site to collect or if no arranged Funeral Director, they will liaise with Seasons Funerals to transfer offsite.
- There is designated fridge in the mortuary for the storage of deceased fetuses.
- In the case of a major incident where a high number of bodies are to be stored, refer to the Emergency Procedure Manual and AKG Business Continuity Plans.
- In the case of a malfunction of the mortuary fridge, the HNM on duty will arrange for transfer of the deceased to pre- approved funeral home and the AKG Business Continuity Plan

11.2 Kalamunda Hospital Transfers

- Kalamunda Hospital nursing staff to page ward PSA to assist with collection of the Bier trolley and transfer of the deceased patient.
- On collection of the Bier trolley, the nurse is to check that the fridge is turned on and no alerts are on the monitor panel. If fridge is not working correctly, then patient is to be collected from ward and alert manager.

- Each of the three mortuary fridges at Kalamunda Hospital has a 300kg capacity.
- Bariatric patients must not be transferred to the body hold fridge unless there are enough staff to facilitate a safe manual handling transfer.
- The nurse is to complete all parts of the sign in section of the Kalamunda Hospital Mortuary Register – See Appendix 1.
- When the Funeral Director arrives, the ward coordinator will page the ward PSA who will, together with a nurse, meet the Funeral Director at the hospital back entrance.
- The nurse must confirm the identity of the deceased patient with the Funeral Director using three points of identification required.
- The nurse and the funeral director are to sign out the deceased in the Kalamunda Hospital Mortuary Register.
- Any discrepancy in identification is to be reported to the NUM in hours, or the Armadale A/H Nurse Manager after hours.
- The nurse is to obtain a collection receipt from the Funeral Director.
- The nurse is to place the receipt in the patient medical record or give to reception staff to arrange for filing with patient's medical record.

11.3 Mortuary viewings (Armada Hospital only)

- Relatives may wish to view the patient in the mortuary. During office hours, the Nurse Unit Manager/Associate Nurse Unit Manager (NUM/ANUM) on the ward/unit will provide staff to assist with viewing. After hours, the HNM on duty is responsible for deciding who the most appropriate person will be to support the family. In most instances, this will be ward staff. Mortuary reviewing should be discouraged where possible unless there is appropriate support available. Timeline for reviewing should be established where possible <12 hours after death.
- Kalamunda Hospital is unable to provide viewing once a patient has been transferred to the Mortuary.

11.4 Bier trolley cleaning

The Bier trolley is to be cleaned and disinfected by the PCA/PSA after each use.

11.5 Armada Mortuary Fridges

- The mortuary fridge is to be cleaned and disinfected by the PCA each time a patient is collected by a Funeral Director.
- In the event of the fridge requiring repairs, the deceased must be transferred to a Funeral Director as ascertained by the HNM and thoroughly cleaned to allow access to the repair personnel.

11.6 Kalamunda and AHS Mortuary Fridges Maintenance

- The mortuary fridge is to be cleaned and disinfected by the PSA each time a patient is collected by a Funeral Director.
 - Remove tray from fridge using the hydraulic lift.
 - Wet tray with approved cleaning/disinfectant product.

- Wipe and ensure all surfaces are evenly wet.
 - Wipe down and roughly dry, then apply product again to cover all surfaces and allow to air dry.
 - Once dry, use hydraulic lift to return tray to fridge. Then clean and disinfect the hydraulic lift.
- The first clean will remove any organic matter and the second application will disinfect the surfaces.
- If there is visible solid matter, a commercial clean is recommended (see below).
- Commercial Clean – every 6 months or as above
 - Every 6 months the fridges need a professional clean for Mould and Bacteria Remediation.
 - This is completed by a commercial cleaner, and avoids PSA staff working in confined spaces, heights etc. The contractor is to supply a compliance certificate attached to the refrigerator after each clean.

12. Morbidity and mortality review

- All deaths are to be reviewed within four months as per the Health Department of Western Australia (HDWA) [Review of Death Policy MP 0098/18](#)

13. Legislative context

The following documents are required to give effect to this procedure:

- [WA Health Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist](#)
- HDWA
 - [Review of Death Policy MP 0098/18](#)
- EMHS
 - [Reporting and Care of the Deceased Policy.](#)

14. Supporting Documentation

The following documents support the implementation of this procedure:

- AKG
 - [Perinatal Loss Guideline](#)
 - [Bariatric Management Procedure](#)
 - [Next of Kin Contact Procedure](#)

15. Compliance, monitoring and evaluation

Compliance against this procedure will be evaluated by consumer feedback from relatives and Funeral Directors (complaints) and audit of mortuary log

16. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 3 – Action 3.6 Standard and transmission-based precautions
- Standard 5 – Action 5.15 Comprehensive care at the end of life
- Standard 5 – Action 5.18

17. Policy owner (relating to these procedures)

Director Nursing and Midwifery

Enquiries relating to this procedure may be directed to:

Title: Coordinator of Nursing
 Department: Hospital wide
 Email: Jo.Harris@health.wa.gov.au

18. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V3	27 11 2025	27 11 2028	Full review, significant changes throughout document

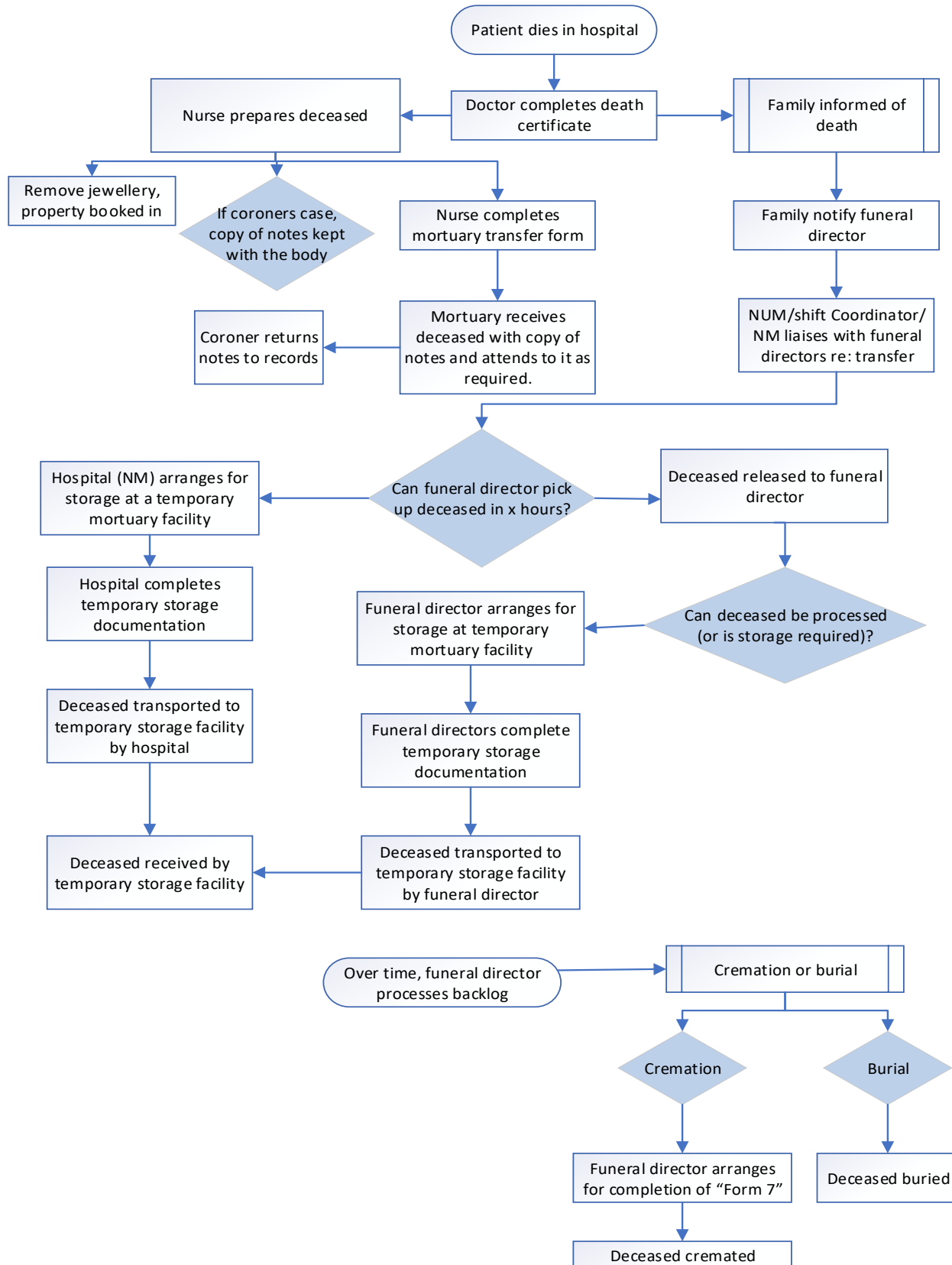
19. Authorisation

This procedure has been authorised and issued by:

Authorised by	Russell Colyer – Cockburn - Director of Nursing and Midwifery
Authorisation date	27 11 2025
Published date	27 11 2025

20. Appendices

20.1 Appendix 1: Pandemic influenza arrangement post death of a patient



20.2 Appendix 2: Armadale Mental Health Service Deceased Client Notification

This process is to ensure that when the Armadale Mental Health Service is advised that a consumer is deceased, that all required notifications are completed, systems and the consumer's health record are processed accordingly.

This process is in addition to required Datix CIMS or Chief Psychiatrist Reporting Form notifications.

